Psychology Training Clinics in Health Service Psychology: Results of the 2025-2026 APTC Annual Survey

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Association of Psychology Training Clinics

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APTC Research Committee

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Executive Summary

The 2026 APTC Annual Survey was completed by **120 clinic directors** representing training clinics across North America. Key findings are summarized below.

Sample. 111 full members (92.5%), 6 associate members, and 1 international affiliate(s) participated. The majority hold Ph.D.s in psychology (89.7%) and have been in their current role for a median of 5.5 years.

Position and compensation. Most directors hold non-tenure-track faculty positions (66.7%). The median gross salary was $104,000. Salary varied substantially by academic rank: assistant professors ($90,000), associate professors ($105,500), and full professors ($137,500). 83.3% reported access to professional development funding (median: $1,838).

Clinic structure. Clinics have operated for a median of 39 years and served a median of 120 clients per year (range: 8–900). 96.9% of clinics offer therapy services and 91.7% offer assessment services. 55% of directors have on-call responsibilities.

Supervision and caseloads. The median maximum therapy caseload per student was 5 cases; the median maximum assessment caseload was 2 cases. Directors who supervise therapy (n = 83) spent a mean of 5.7 hours per week, compared to 3.7 hours among those who supervise assessment (n = 54); see Section 4.4 for the within-person comparison among directors who supervise both and a discussion of measurement considerations underlying this gap.

Finances. The median annual clinic revenue was $43,726. 89.4% of clinics offer a sliding fee scale. 12.5% bill insurance.

Technology. 70% of clinics use an electronic health record system. 70.8% offer telehealth. 25.3% of clinic directors personally use AI in their work; formal AI policies are in place at 76.9% of responding clinics.

Well-being. Directors reported a mean composite burnout score of 2.24 on a 0–6 scale (*SD* = 1.26). Qualitative themes reflect that colleagues, autonomy, and meaningful impact sustain directors, while administrative burden and lack of institutional recognition are primary challenges.

Full results, including figures and tables for each domain, appear in the sections that follow.

Introduction

The APTC Annual Survey has been administered annually by the APTC Research Committee to document the current state of health service psychology training clinics across North America. The survey serves a practical purpose: giving clinic directors data they can use to contextualize their own clinic's operations, advocate for resources, and benchmark against their peers. The survey was administered via Qualtrics between December 2025 until February 2026 to all APTC member clinics.

This report presents results from **120 respondents** — 111 full members (92.5%), 6 associate members, and 1 international affiliate(s). Throughout this report, *n*s vary by item because not all respondents answered every question. Results are presented as frequencies, percentages, and distributional summaries (means, medians, ranges). For salary and sensitive subgroup data, cells based on fewer than three respondents are suppressed and displayed as "—" to protect respondent privacy. Most questions pertaining to numbers refer to LAST clinic year (August 2024- July 2025) to allow for revenue alignment, allow respondents to rely on existing data rather than estimating future data, and for consistency across clinics.

A **clinic size taxonomy** is used throughout to stratify results where meaningful: clinics are classified as **Small** (< 93 clients/year), **Medium** (93–130 clients/year), **Large** (131–250 clients/year), or **Giant** (> 250 clients/year), based on annual client volume. These cutoffs were established in the 2025 cycle and are retained here for comparability.

> **A note on *n*s and interpretation**
>
> Because survey completion was not required for every item, the effective sample size varies throughout this report. All percentages are computed from valid (non-missing) responses to that item unless otherwise noted.

1 Who Responded: Sample Characteristics

1.1 Respondent Background

The majority of 2026 survey respondents hold doctoral degrees in psychology: 89.7% hold a Ph.D. (n = 105) and 8.5% hold a Psy.D. (n = 10). Directors have been in their current role for a median of 5.5 years (M = 7.7, SD = 6.6; range: 1–31 years), indicating that most respondents have substantial experience leading their clinics.

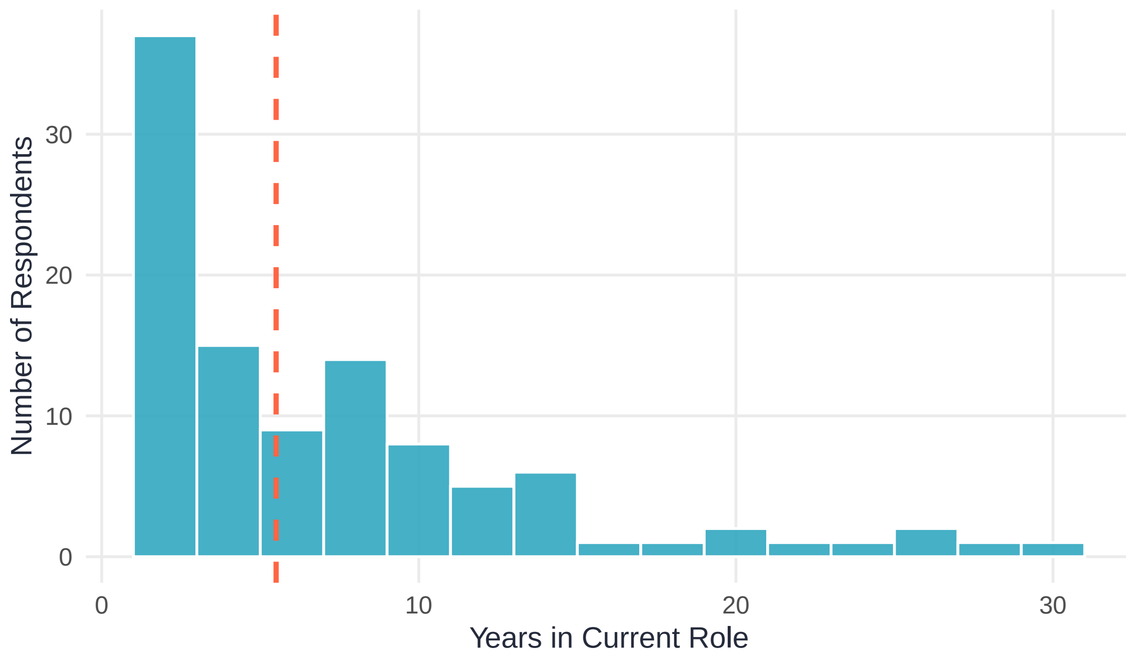

Figure 1. *Distribution of years in the clinic director role. The dashed line indicates the median.*

Table 1. *Professional Degree of Clinic Directors*

Degree	n	%
PhD	105	89.7%
PsyD	10	8.5%
Masters level	1	0.9%
Other (please specify):	1	0.9%

1.2 Institutional and Geographic Context

Clinics are situated across a range of community settings: 47.4% are based in urban areas and 36.2% in non-metro communities, with smaller proportions in suburban and rural settings. However, the populations clinics serve frequently extend beyond their own immediate setting. 15.7% of directors described their client base as mixed or balanced across multiple area types rather than concentrated in one (Figure 2), and average client composition data reinforce this: most clinics report drawing clients from more than one type of community, regardless of where the clinic itself is physically located.

Table 2. *Geographic Region of Clinic*

Region	n	%
U.S: South, South Atlantic (DE, DC, FL, GA, MD, NC, SC, VA, WV)	24	20.7%
U.S: Midwest, East North Central (IL, IN, MI, OH, WI)	20	17.2%
U.S: West, Pacific (AK, CA, HI, OR, WA)	14	12.1%
U.S: South, East South Central (AL, KY, MS, TN)	12	10.3%
U.S: West, Mountain (AZ, CO, ID, MT, NV, NM, UT, WY)	11	9.5%
U.S: Northeast, Middle Atlantic (NJ, NY, PA)	9	7.8%
U.S: Midwest, West North Central (IA, KS, MN, MO, NE, ND, SD)	8	6.9%
U.S: South, West South Central (AR, LA, OK, TX)	8	6.9%
U.S: Northeast, New England (CT, ME, MA, NH, RI, VT)	6	5.2%
Canada	3	2.6%
International	1	0.9%

Table 3. *Immediate Area Where Clinic Is Located*

Clinic Setting	n
Urban (city with population greater than 250,000)	55
Non-Metro Community (town, village, or small city not part of a metro area, with a population between 2,500 and 250,000)	42
Suburban (within an urban metro area, but not within its city limits)	17
Rural (population of less than 2,500)	2

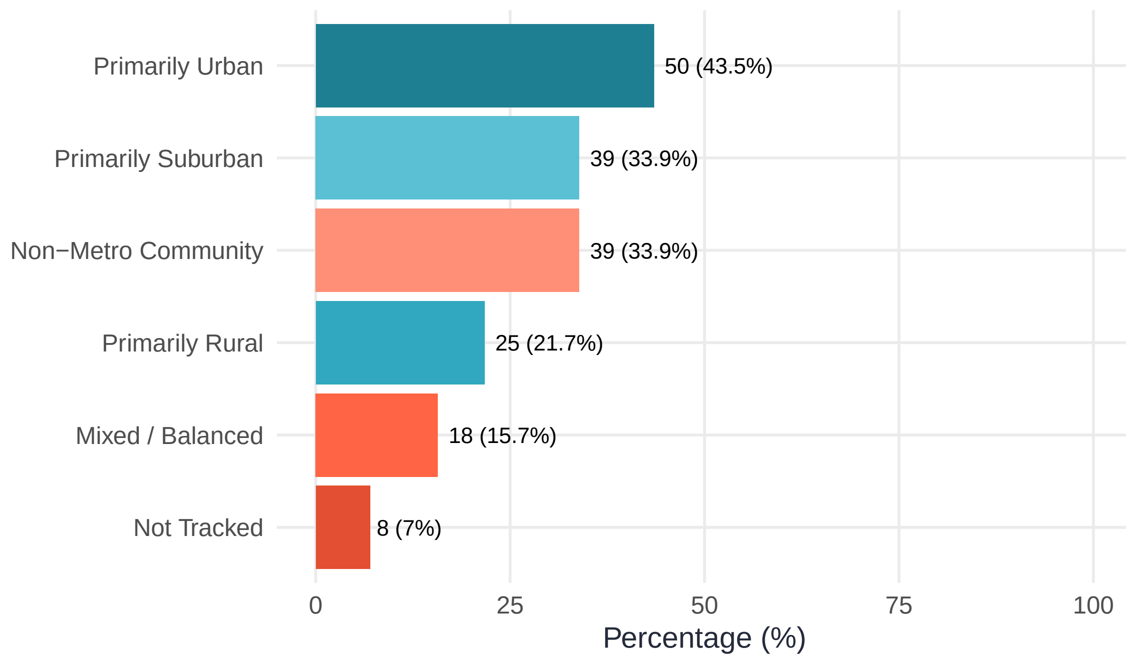

Figure 2. *Area where most clinic clients live. Directors could select multiple options.*

Table 4. *Average Percentage of Clinic Clients by Area Type*

Area	n	Mean %	Median %
Urban	89	39.5	35
Non-Metro Community	89	26.0	10
Suburban	89	24.3	15
Rural	89	10.2	5

Table 5. *Institutional Type and Carnegie Classification*

University Type	Carnegie Classification	n	%
Public	Very High Research Activity (R1)	63	52.5%
Private	Very High Research Activity (R1)	16	13.3%
Public	High Research Activity (R2)	14	11.7%
Private	Doctoral/Professional Universities (D/PU)	11	9.2%
Public	Other (please specify)	4	3.3%
Private	High Research Activity (R2)	3	2.5%
Public	Doctoral/Professional Universities (D/PU)	2	1.7%
Public	Master's Colleges & Universities	2	1.7%
Private	Other (please specify)	1	0.8%

2 Position Characteristics

2.1 Appointment Type and Tenure Status

Clinic directors work across a range of appointment structures. The largest group holds non-tenure-track faculty positions ($n = 80$; 66.7%), followed by administrative or staff positions ($n = 19$; 15.8%), and tenure-track or tenured faculty ($n = 14$; 11.7%).

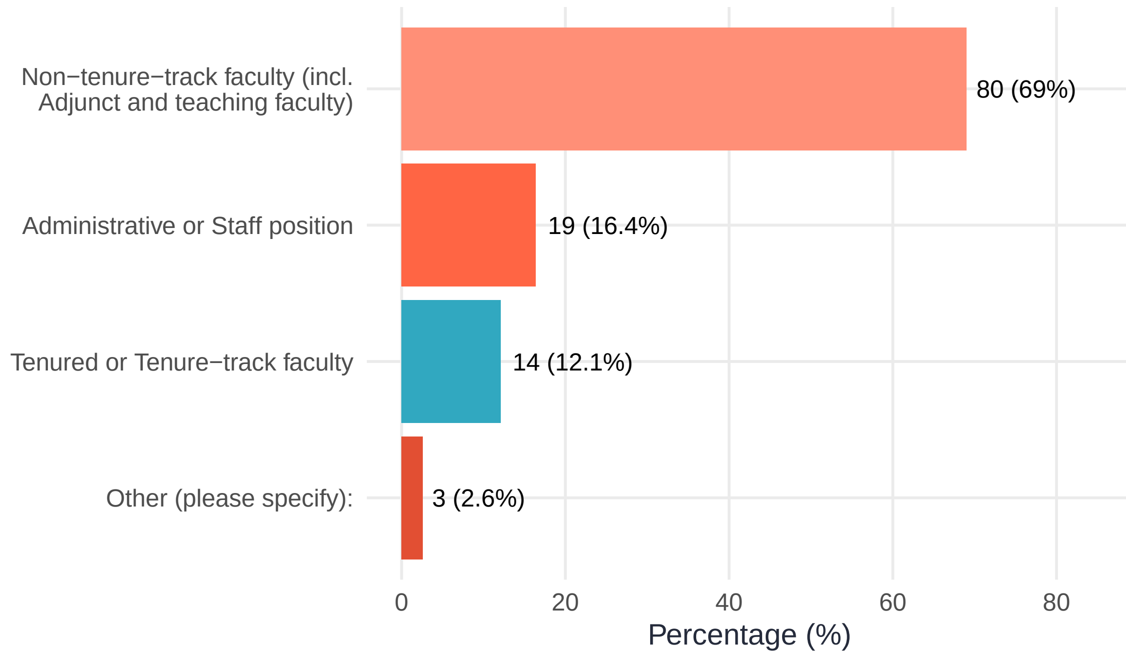

Figure 3. Distribution of appointment types among clinic directors.

2.2 Academic Rank

Among those with faculty appointments, academic rank distributions are shown below. Cells based on fewer than three respondents are suppressed.

Table 6. Academic Rank by Tenure/Appointment Status

Academic Rank	Non-Tenure-Track	Staff / Admin / Other	Tenure-Track / Tenured
Assistant Professor	26	1	5
Associate Professor	34	0	6
Full Professor	10	1	3
Lecturer	1	0	0
Staff / Admin	0	13	0
Other	7	0	0

2.3 Who Directors Report To

Directors most commonly report to their department chair (63.3%). The figure below shows the full distribution of reporting structures.

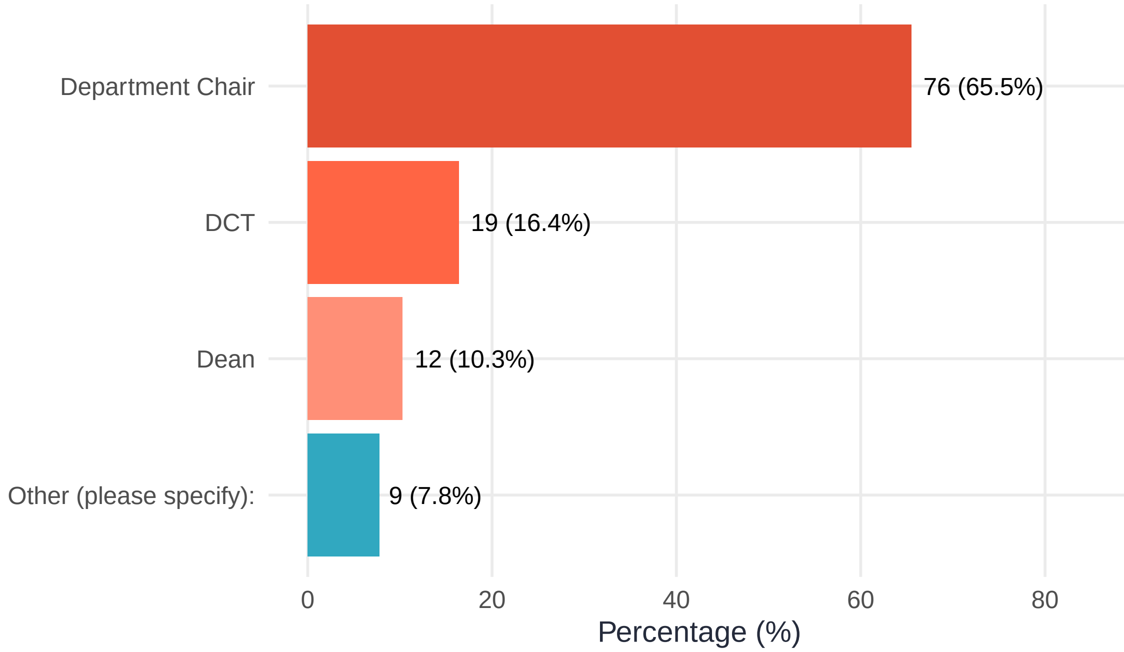

Figure 4. *Administrator or position to whom clinic directors report.*

2.4 Contract Characteristics

Table 7. *Contract Duration (months per year)*

Contract Duration	n	%
12-months	79	68.7%
9-months (academic-year)	18	15.7%
Other (please specify)	9	7.8%
11-months	7	6.1%
10-months	2	1.7%

2.5 Associate / Assistant Directors

19.2% of respondents (n = 23) reported having an associate or assistant director in their clinic.

3 Salary and Compensation

3.1 Overall Salary Distribution

The median gross salary across all responding clinic directors was $104,000 ($M$ = $106,412, SD = $31,153; range: $11,000–$210,000). As has been consistently true in prior years, salary varied substantially across appointment types and academic rank.

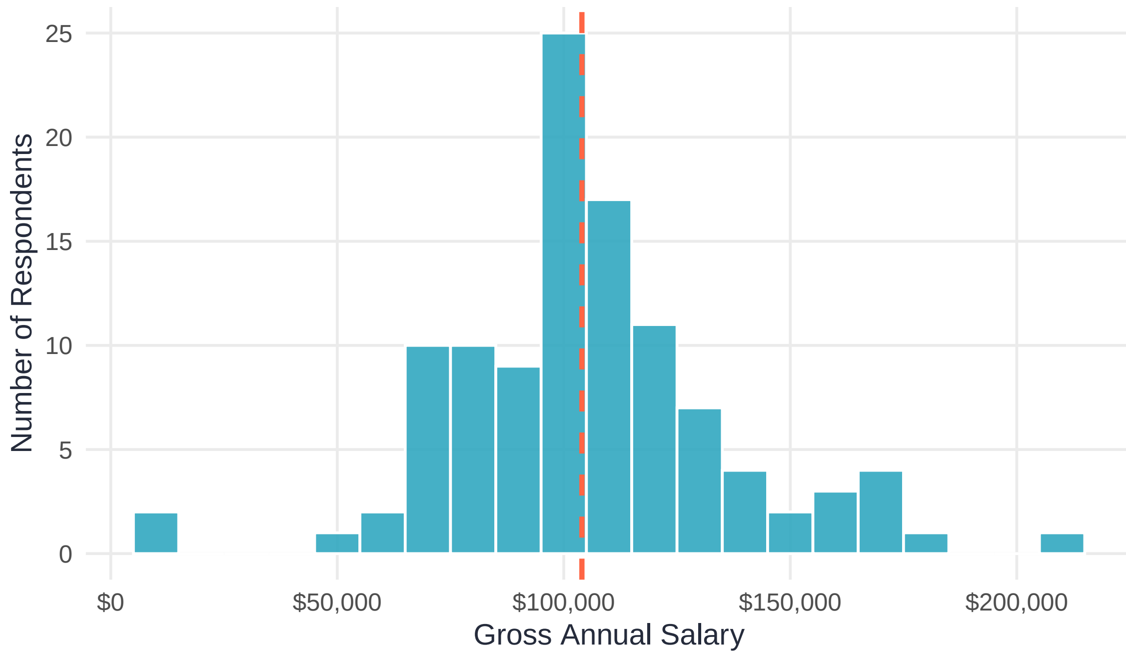

Figure 5. Distribution of gross annual salaries among clinic directors. The dashed vertical line indicates the median.

3.2 Salary by Academic Rank

Median salaries increased with academic rank. Among assistant professors, the median salary was $90,000; among associate professors, $105,500; and among full professors, $137,500. Directors in staff or administrative positions reported a median salary of $120,000.

Table 8. *Gross Annual Salary by Academic Rank (cells suppressed if $n < 3$)*

Rank / Role	n	Median Salary	Mean Salary	SD
Assistant Professor	31	$90,000	$88,429	$21,968
Associate Professor	38	$105,500	$112,983	$26,500
Full Professor	12	$137,500	$135,333	$32,052
Lecturer	1	—	—	—
Staff / Admin	13	$120,000	$124,064	$27,548

3.3 Salary by Appointment Type

Table 9. *Gross Annual Salary by Appointment Type (cells suppressed if n < 3)*

Appointment Type	n	Median Salary	Mean Salary	SD
Non-Tenure-Track	75	$100,826	$104,072	$29,165
Staff / Admin / Other	22	$116,416	$113,724	$35,023
Tenure-Track / Tenured	12	$101,744	$107,626	$36,316

3.4 Salary by Contract Length

Table 10. *Gross Annual Salary by Contract Length (cells suppressed if n < 3)*

Contract Length	N	Mean	Median	SD
3 years	33	$105,186	$102,204	$25,204.18
Yearly	29	$99,457	$100,000	$28,051.72
Tenured/permanent/unlimited	25	$111,107	$110,000	$35,835.18
Other (please specify):	18	$116,322	$111,500	$38,705.03
2 years	4	$93,000	$90,000	$23,636.13

3.5 Salary by Contract Type

Table 11. *Gross Annual Salary by Contract Type (cells suppressed if n < 3)*

Contract Type	n	Median	Mean	SD
12-months	73	$110,000	$112,651	$32,075
9-months (academic-year)	17	$89,000	$88,008	$15,552
Other (please specify)	9	$110,000	$117,000	$20,958
11-months	7	$86,000	$91,227	$17,687
10-months	2	—	—	—

Table 12. *Gross Annual Salary by Contract Type and Academic Rank (cells suppressed if n < 3)*

Contract Type	Rank / Role	n	Median	Mean	SD
12-months	Assistant Professor	18	$95,798	$97,086	$17,149
12-months	Associate Professor	26	$114,000	$120,283	$28,414
12-months	Full Professor	9	$151,000	$142,333	$33,808
12-months	Lecturer	1	—	—	—
12-months	Staff / Admin	9	$120,000	$122,426	$29,576
9-months (academic-year)	Assistant Professor	8	$78,059	$82,253	$14,357
9-months (academic-year)	Associate Professor	5	$97,846	$99,502	$11,025
9-months (academic-year)	Full Professor	1	—	—	—

3.6 Salary by Region

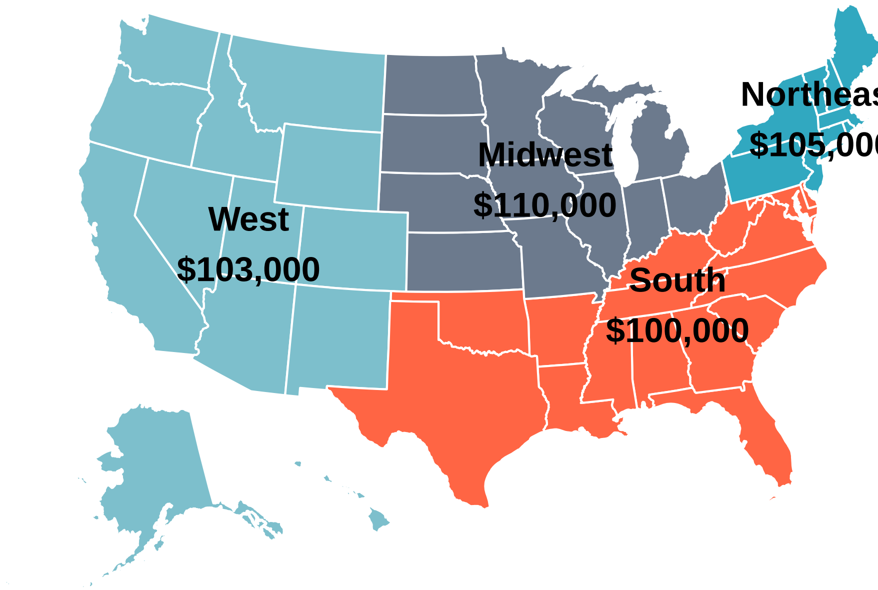

Figure 6. *Median gross annual salary by U.S. census region. Regional medians are based on survey respondents within each region.*

3.7 Salary Funding Sources

Most directors' salaries are funded primarily through their department or university. The figure below shows the proportion of directors who endorsed each funding source (multiple selections permitted).

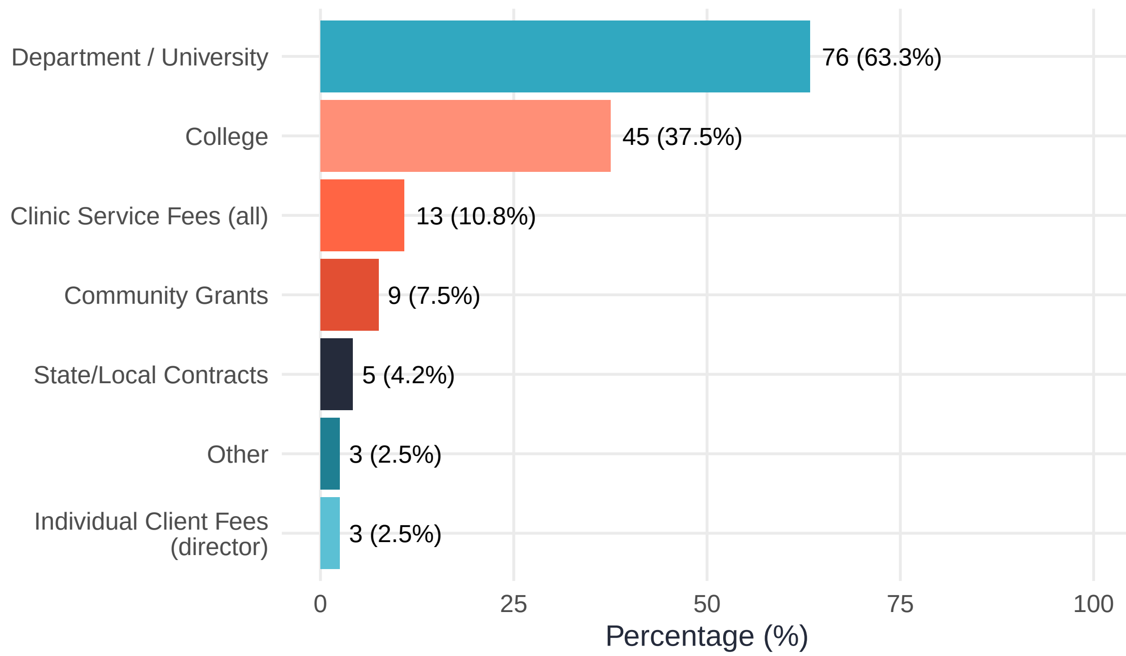

Figure 7. *Funding sources for clinic director salaries. Directors could select multiple sources.*

3.8 Professional Development Funding

83.3% of directors (n = 90) reported having access to professional development funds. Among those who reported a specific dollar amount, the median was $1,838 ($M$ = $2,004).

Table 13. *Access to Professional Development Funding*

PD Funding Access	n	%
Yes	90	83.3%
No	18	16.7%

3.9 Additional Compensation

Directors were asked about compensation beyond their base salary, including stipends, course releases, and summer salary. The figure below shows the prevalence of each type of additional compensation. Note that a survey design issue allowed multiple responses where single selection was intended; these data should be interpreted with caution.

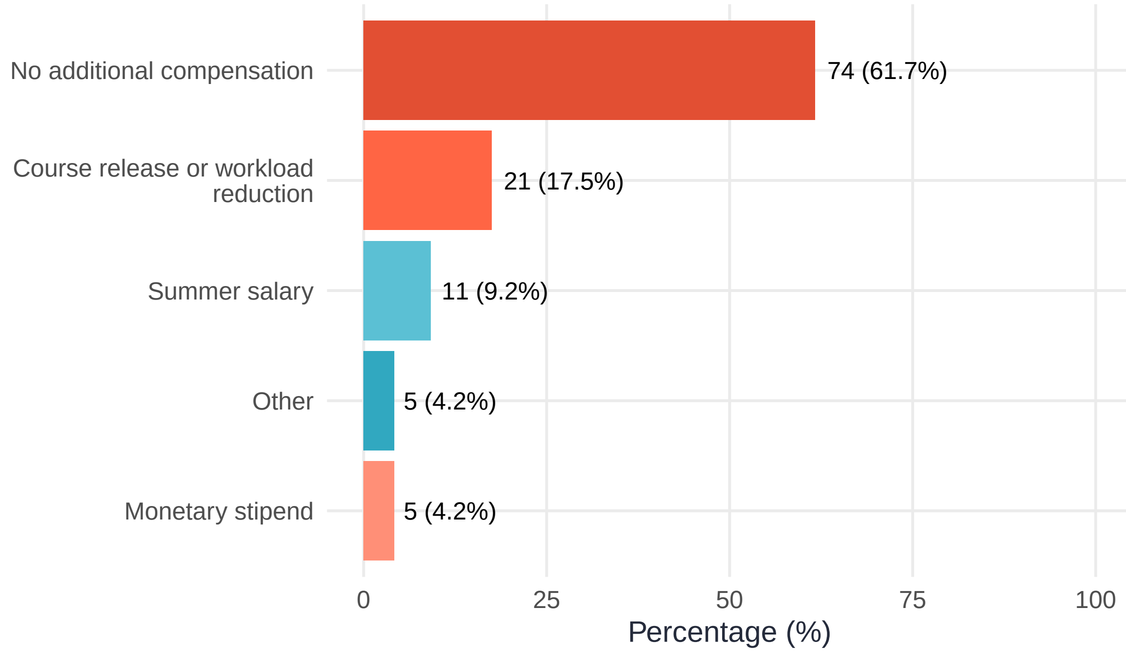

Figure 8. *Forms of additional compensation reported by clinic directors. Directors could select multiple types.*

Among the small number of directors reporting a monetary stipend (n = 5), the median stipend amount was $10,000. Among those reporting summer salary (n = 11), the median amount was $10,000.

4 Responsibilities and Workload

4.1 Director Responsibilities

Directors engaged in a wide range of professional activities. Nearly all reported responsibilities in administration (91.7%) and clinical supervision (90.8%). The figure below shows the full distribution of responsibilities.

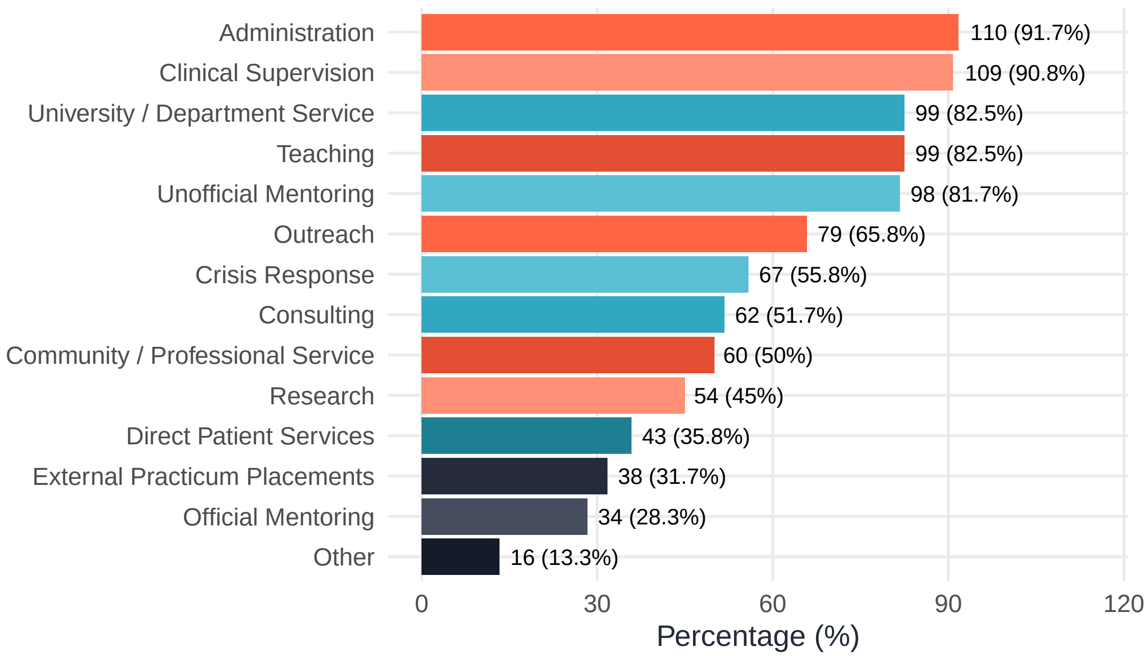

Figure 9. Professional responsibilities of clinic directors. Directors could select multiple roles.

4.2 On-Call Responsibilities

55% of directors (n = 66) reported having some on-call responsibilities.

4.3 Teaching

65.8% of directors (n = 79) reported teaching at least one course. Among those who teach, the median number of courses per year was 2 (range: 1–8).

4.4 Hours Spent on Clinic Activities

Directors spend a substantial portion of their week on supervision-related activities. Among directors who supervise therapy (n = 83), mean weekly hours were 5.7 (median = 5). Among directors who supervise assessment (n = 54), mean weekly hours were 3.7 (median = 3). Among the 48 directors who supervise both, mean weekly hours were 5.4 for therapy versus 3.6 for assessment (medians: 4 and 3, respectively) — the same within-person gap observed at the aggregate level.

The lower hours reported for assessment supervision should be interpreted with some caution. The survey item did not specify whether "hours spent supervising" was meant to include or exclude time spent reviewing or providing feedback on written reports, which may be more substantial for assessment than for therapy. Consistent with this possibility, hours supervising assessment correlated moderately with hours spent reviewing documentation ($r = 0.37$, $n = 54$), a notably stronger relationship than the corresponding correlation for therapy ($r = 0.15$, $n = 83$). This pattern is consistent with — though does not confirm — the possibility that some directors' assessment supervision estimates reflect direct contact time only, with report-writing time captured separately under documentation. Future survey cycles may consider clarifying this distinction in the item wording.

Table 14. *Hours per Week Spent on Clinic-Related Activities*

Activity	n	Mean (hrs)	SD	Median (hrs)	Range
Supervising Therapy	83	5.7	3.8	5	1–20
Reviewing Documentation	90	4.1	3.9	3	1–25
Supervising Assessment	54	3.7	3.2	3	0–20
Reviewing Session Recordings	90	3.1	3.0	2	0–20
Supervision-of-Supervision	89	1.6	2.1	1	0–15

5 Clinic Structure and Operations

5.1 Clinic Characteristics

The clinics represented in this survey have been operating for a median of 39 years (M = 35.6 years). On average, clinics served 20 trainees (range: 1–80) and 120 clients per year (range: 8–900).

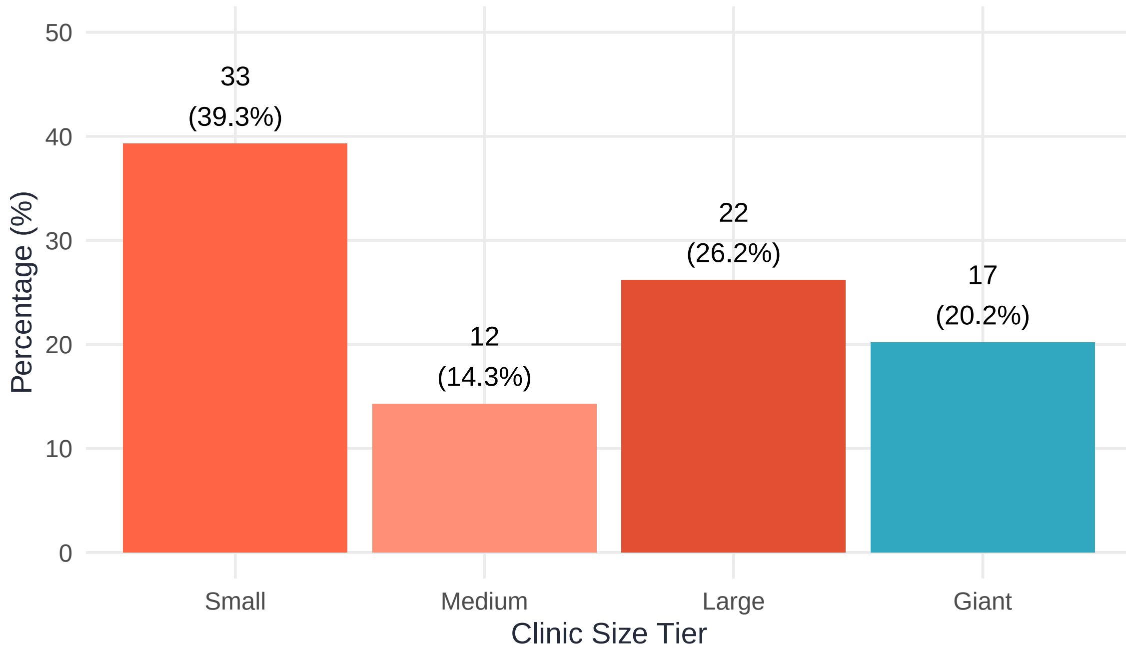

Figure 10. *Distribution of clinics by size tier, defined by annual client volume. Clinics are classified as Small (< 93 clients/year), Medium (93–130 clients/year), Large (131–250 clients/year), or Giant (> 250 clients/year), based on annual client volume.*

5.2 Training Model

Table 15. *Primary Training Model*

Training Model	n	%
Scientist-Practitioner	51	53.1%
Clinical Science	29	30.2%
Scholar-Practitioner	10	10.4%
Practitioner-Scientist	4	4.2%
Other (please specify):	2	2.1%

5.3 Programs and Student Composition

Directors reported a median of 20 trainees in the current academic year (M = 23.4; range: 1–80), mostly consistent with their values in past years — among the 88 directors who reported totals for both years, counts were strongly correlated ($r = 0.97$) and typically differed by only 1 trainee(s). Trainee counts dropped notably over the summer: among 93 directors reporting summer (2025) totals, the median was 16 trainees, and 8.6% (n = 8) reported no trainees at all during that period. The section tables below summarize the types of programs supported across responding clinics and provide additional detail on past-year totals by clinical domain.

Table 16. *Training Programs Supported by Clinics (Select All That Apply)*

Training Program	n	%
Clinical PhD	69	57.5%
Counseling PhD	18	15%
Clinical PsyD	15	12.5%
School PhD	15	12.5%
Terminal Master's program	13	10.8%
Other (please specify):	10	8.3%
School EdS	3	2.5%

Trainee and supervisor counts also varied by clinical domain, as reported in the section tables below. A small number of responses include values that exceed plausible bounds relative to other fields in the same response (e.g., a reported maximum of 400 therapy trainees), which are included here without correction or exclusion.

Table 17. *Trainee and Supervisor Counts, Last Clinic Year (August 2024–July 2025)*

Label	n	Median	Min	Max
Assessment Supervisors	84	3	0	15
Assessment Trainees	81	12	0	50
Therapy Supervisors	89	4	0	309
Therapy Trainees	89	17	0	400
Total Trainees	89	20	4	80

39.6% of clinics (n = 36 of 91) reported serving students from more than one academic program. This aligns closely with the number of clinics that went on to describe a second program in detail (n = 36), though slightly fewer fully detailed a third program (n = 9 of the 11 reporting three or more), suggesting modest drop-off in completing the more granular program-level items. Each program was described individually for the last clinic year (August 2024–July 2025), including student counts and how clinic caseload split between therapy and assessment. Across 137 individually described programs, the tables below summarize program type, typical student counts, caseload distribution, and primary clinical focus by program type. Therapy/assessment percentages are intended to sum to roughly 100% across a clinic's programs; 4 of the clinics with multiple programs reported totals noticeably above 100%, which are included here as collected.

Table 18. *Program Type Across All Individually Described Programs*

Program Type
Clinical Psychology
Other (please specify)
Counseling Psychology
School Psychology
Combined Program (select ONLY if the program itself is designed to train in multiple disciplines, not if your department has separ

Table 19. *Student Counts and Caseload Share by Program Type, Last Clinic Year (August 2024–July 2025)*

Program Type
Clinical Psychology
Other (please specify)
Counseling Psychology
School Psychology
Combined Program (select ONLY if the program itself is designed to train in multiple disciplines, not if your department has separ

Table 20. *Primary Clinical Focus by Program Type*

Program Type
Clinical Psychology
Other (please specify)
Counseling Psychology
School Psychology
Combined Program (select ONLY if the program itself is designed to train in multiple disciplines, not if your department has separ

5.4 Services Offered

Of those who responded to questions about services offered, 96.9% of clinics (n = 96) offer therapy services and 91.7% (n = 96) offer assessment services.

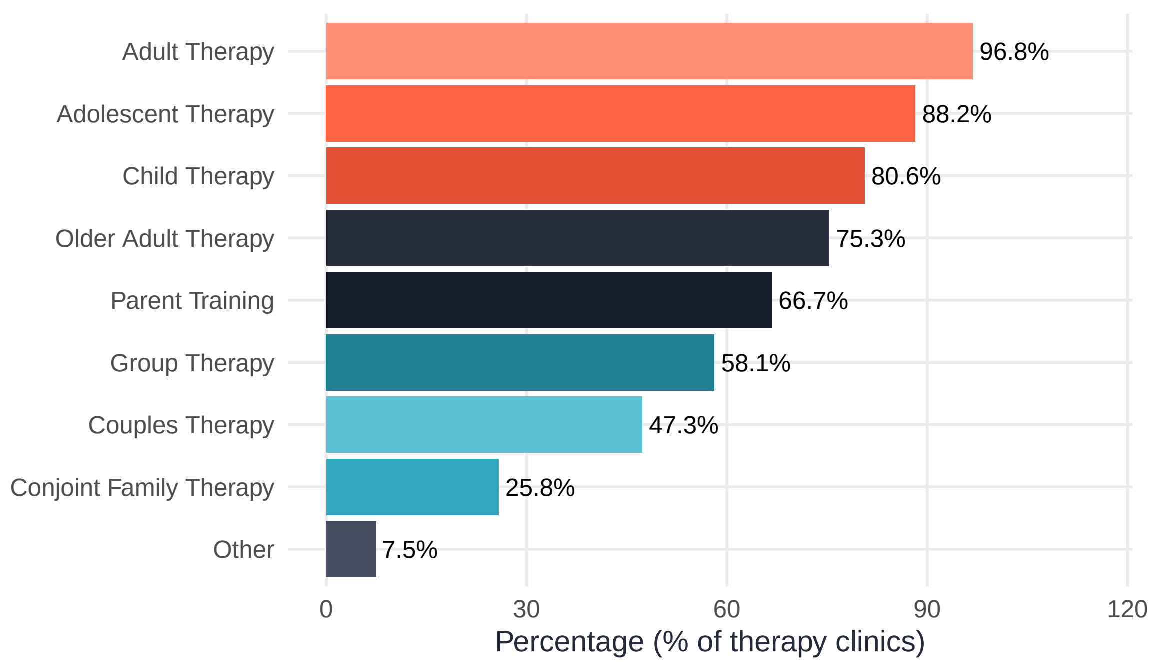

Figure 11. *Therapy service types offered. Among clinics providing therapy.*

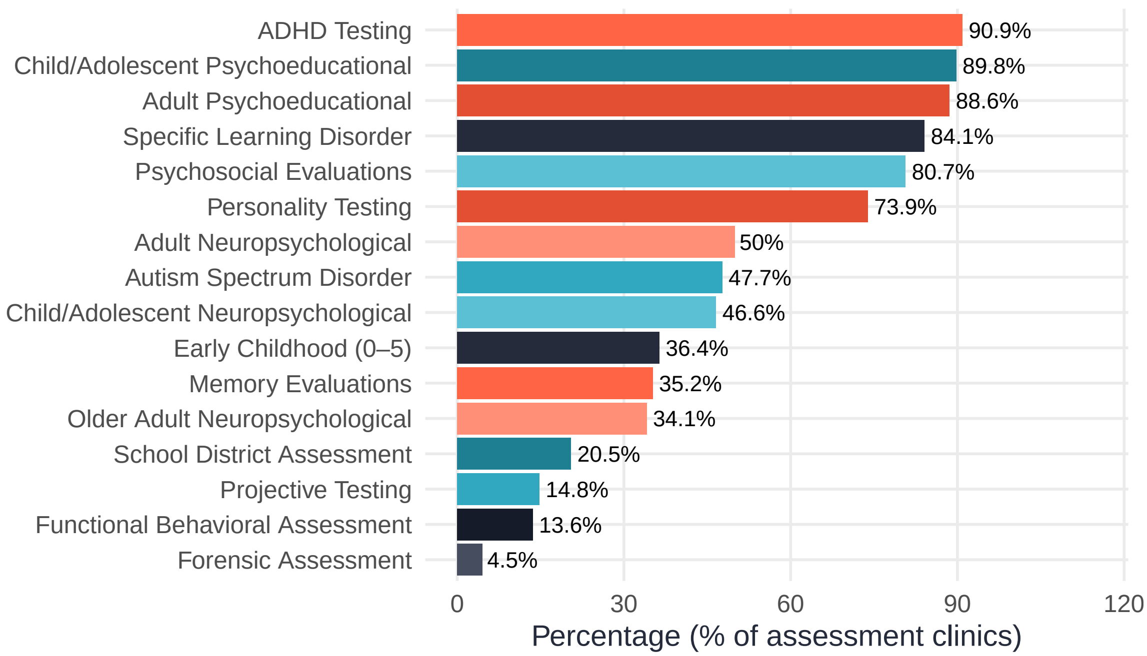

Figure 12. *Assessment service types offered. Among clinics providing assessment.*

5.5 Clinic Hours and After-Hours Coverage

Table 21. *Clinic Operating Hours per Week*

n	Mean	SD	Median	Min	Max
87	43	11.9	42	12	70

Table 22. *After-Hours Service Coverage (Select All That Apply)*

After-Hours Coverage	n	%
Yes, extended weekday hours. Please specify what hours are offered:	72	60%
Other: Occasionally on a case by case basis or depends on service type. Please explain:	13	10.8%
No	12	10%
Yes, weekend hours. Please specify what hours are offered:	12	10%

6 Supervision and Student Training

6.1 Supervisor Composition

65% of clinics (n = 78) require their supervisors to be licensed. 56.7% of clinics (n = 51) reported using community or external supervisors. The breakdown of supervision by role during the academic year is shown below.

Table 23. *Percentage of Supervision Provided by Role (Academic Year)*

Role	n	Mean %	Median %
Clinic Director	91	34.3	25
Program Faculty	91	26.2	20
Community/External (paid)	91	14.5	0
Community/External (unpaid)	91	12.6	0
Other	91	6.6	0
Other Clinic Staff	91	5.8	0

Supervisory teams also varied by clinical domain over the last clinic year (August 2024–July 2025), summarized below. 3 responses reported more supervisors overseeing both domains than the smaller domain's total supervisor count. These responses were retained for the raw therapy, assessment, and overlap counts, but were excluded from the derived total unique-supervisor statistic.

Table 24. *Supervisor Counts by Domain, Last Clinic Year (August 2024–July 2025)*

Label	n	Median	Min	Max
Assessment Supervisors	84	3	0	15
Supervisors Overseeing Both Domains	83	1	0	50
Therapy Supervisors	89	4	0	309
Total Unique Supervisors (Tx + Ax − Overlap)	77	5	0	311

6.2 Student Caseloads

Typical maximum caseloads varied by clinical domain. Therapy students were permitted to carry up to a median of 5 cases at one time (M = 5.6; range: 2–10; n = 77), while assessment students carried a median maximum of 2 cases (M = 2.4; range: 1–8; n = 79). Trainee counts showed little relationship to these per-student caseload limits (n = 74; therapy: r = -0.18; assessment: r = -0.11), suggesting that caseload policies are generally set independent of overall staffing levels.

Table 25. *Student Caseload Limits and Supervisory Team Sizes*

Metric	n	Mean	SD	Median	Min	Max
Max assessment cases per student (policy)	79	2.4	1.2	2	1	8
Max assessment trainees per supervisor	77	6.3	3.9	5	1	25
Max therapy cases per student (policy)	77	5.6	2.1	5	2	10
Max therapy trainees per supervisor	83	7.4	4.0	6	1	30

6.3 Supervisor Caseloads

While the previous section described how many cases an individual student is permitted to carry, supervisors are typically responsible for several students' caseloads at once. Over the last clinic year (August 2024–July 2025), therapy supervisors oversaw a median of 17.7 clients each ($n = 79$), compared to 12.5 clients per assessment supervisor ($n = 76$) — a notably larger supervisory caseload for therapy than assessment. The table below breaks this down by clinic size tier.

Table 26. *Clients per Supervisor by Domain and Clinic Size Tier, Last Clinic Year (August 2024–July 2025)*

Clinic Size	n (Therapy)	Median Clients per Tx Supervisor	n (Assessment)	Median Clients per Ax Supervisor
Small	26	14.6	27	10.5
Medium	12	18.3	10	10.4
Large	19	20.0	17	12.5
Giant	17	29.5	15	30.7

6.4 How Caseloads Are Determined

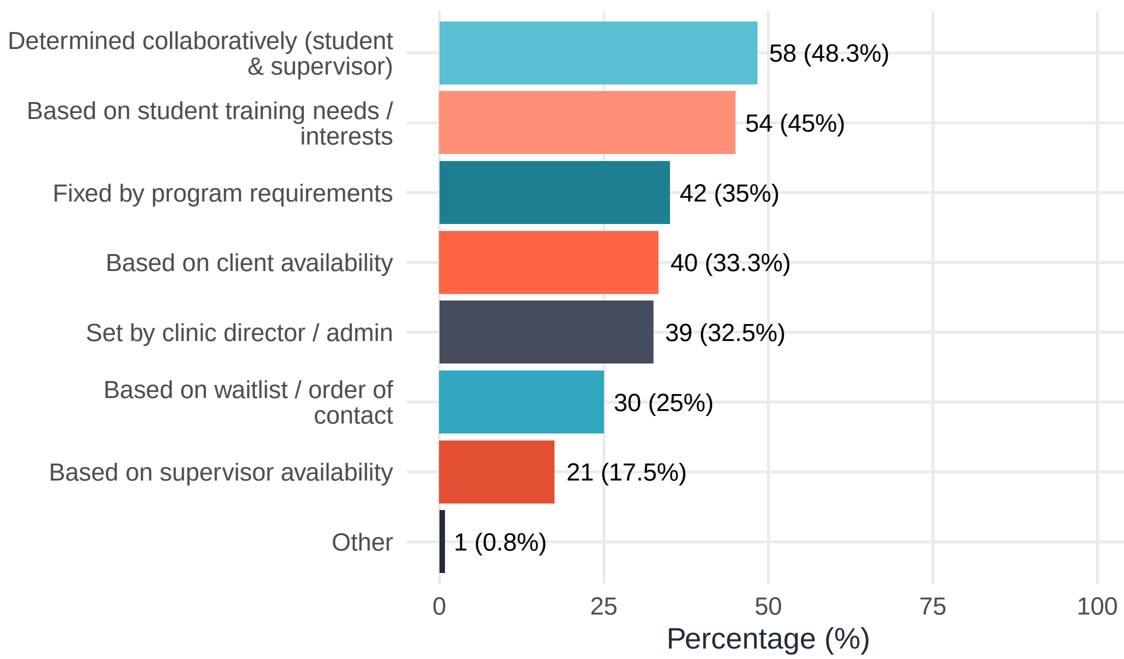

Figure 13. *Methods by which student caseloads are determined. Multiple selections permitted.*

6.5 Clinic Size and Training Capacity

The table below shows key training metrics stratified by clinic size tier.

Table 27. Training Metrics by Clinic Size Tier. Small < 93 clients/year, Medium = 93-130 clients/year, Large = 131-250 clients/year, or Giant > 250 clients/year.

Clinic Size	n	Median Trainees	Median Clients	Median Tx Supervisors	Median Ax Supervisors
Small	33	15.0	63.0	3.0	2.0
Medium	12	25.0	110.5	4.5	3.0
Large	22	22.5	158.0	6.0	3.5
Giant	17	30.0	305.0	8.0	3.0

7 Staff and Facilities

7.1 Staff Composition

56.7% of clinics employed professional support staff ($n = 68$), 50% employed graduate assistants ($n = 60$), 29.2% employed undergraduate students in clinic support roles ($n = 35$), and 6.7% had postdoctoral fellows ($n = 8$). Among clinics with each staff type, the following table summarizes typical staffing levels — note that these are reported in different units (total FTE, total weekly hours, or headcount) depending on staff type, as collected in the survey.

Table 28. *Clinic Staffing Summary (Among Clinics With Each Staff Type)*

Label	n	Median	Mean	Min	Max
Graduate Staff (Total FTE)	60	0.75	1.0	0.125	7
Postdocs (Headcount)	8	2.50	2.5	1.000	4
Support Staff (Total FTE)	68	1.00	1.6	0.050	20
Undergraduate Staff (Total Hours/Week)	35	16.00	27.1	3.000	210

7.2 Physical Space

Table 29. *Clinic Spaces and Facilities (among 90 respondents; select all that apply)*

Space Type	n	%
Client waiting room	83	92.2%
Student workrooms or shared office space	82	91.1%
Director's office	79	87.8%
Multipurpose rooms	77	85.6%
Reception or check-in area	75	83.3%
Storage rooms	73	81.1%
Dedicated therapy rooms	59	65.6%
Group therapy rooms	50	55.6%
Observation rooms	49	54.4%
Dedicated assessment/testing rooms	40	44.4%
Supervisor office space	34	37.8%
Other	34	37.8%
Assistant director's office	25	27.8%

Clinics reported a range of physical spaces to support clinical service delivery, supervision, and student training. Most clinics reported having a client waiting room ($n = 83$) and a reception or check-in area ($n = 75$). Dedicated therapy rooms were reported by 59 clinics, and dedicated assessment/testing rooms were reported by 40 clinics.

Across clinics reporting total room counts, clinics had a mean of 7.3 rooms (median = 8; middle 50%: 6–9; range: 3–10; $n = 29$). Among clinics reporting the number of dedicated therapy rooms, the mean was 6.7 rooms (median = 6; middle 50%: 4–9; range: 1–18; $n = 57$). Among clinics reporting the number of dedicated assessment/testing rooms, the mean was 2.3 rooms (median = 2; middle 50%: 1–3; range: 1–6; $n = 34$). Clinic square footage was reported by 53 respondents; the most common category was 2,000–4,999 sq ft ($n = 19$).

8 Fees, Finances, and Revenue

8.1 Fee Structures

Table 30. Fee Structures for Therapy and Assessment Services

Service Type	n	Mean	Median	Min	Max
Assessment Maximum	76	$597	$500	$6	$3,000
Assessment Minimum	77	$328	$225	$0	$3,000
Assessment Typical	77	$1,004	$800	$20	$3,500
Therapy Maximum	79	$25	$15	$0	$154
Therapy Minimum	81	$12	$5	$0	$154
Therapy Typical	81	$66	$50	$10	$260

The median therapy session fee across clinics was $50 (range of medians: minimum $5 – maximum $15). For assessment, the median typical fee was $800 (range of medians: $225 – $500).

8.2 Sliding Scale Fees

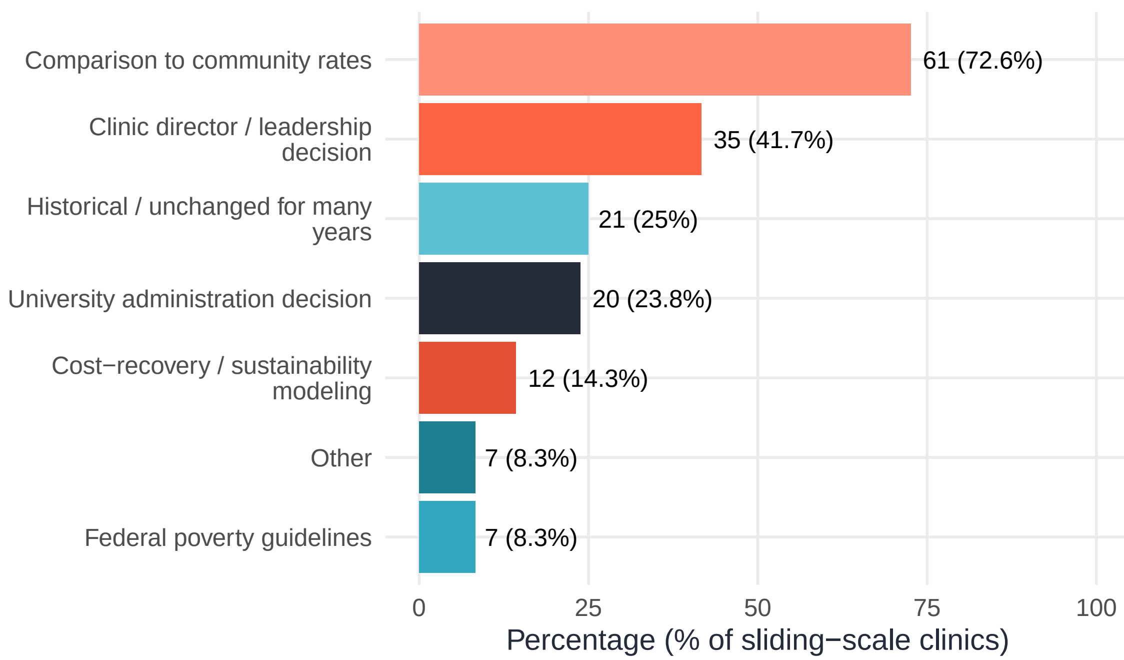

Figure 14. Basis for determining therapy fee amounts shown; the breakdown for assessment was similar. Multiple selections permitted.

Table 31. *Year Fees Were Last Adjusted (Therapy and Assessment)*

Service	Year Last Adjusted	n	%
Assessment	2024–2025	31	38.3%
Assessment	2020–2023	19	23.5%
Assessment	2015–2019	12	14.8%
Assessment	Before 2015	2	2.5%
Assessment	Before 2000	1	1.2%
Assessment	Never changed fees	7	8.6%
Assessment	Unknown	9	11.1%
Therapy	2024–2025	28	33.3%
Therapy	2020–2023	17	20.2%
Therapy	2015–2019	14	16.7%
Therapy	Before 2015	1	1.2%
Therapy	Before 2000	3	3.6%
Therapy	Never changed fees	4	4.8%
Therapy	Unknown	17	20.2%

Table 32. *Direction of Most Recent Fee Changes for Therapy and Assessment*

Service	Direction	n	%
Assessment	Increased	49	77.8%
Assessment	Unknown	10	15.9%
Assessment	Decreased	4	6.3%
Therapy	Increased	47	74.6%
Therapy	Decreased	8	12.7%
Therapy	Unknown	8	12.7%

89.4% of clinics reported offering a sliding fee scale. Among clinics with sliding scale fee structures, the figure above shows how therapy fee amounts are determined. Assessment fee determination followed a broadly similar pattern.

8.3 Insurance Billing

Table 33. *Does the Clinic Bill Insurance?*

Bills Insurance?	n	%
No	77	87.5%
Yes	11	12.5%

12.5% of clinics ($n = 11$) reported billing insurance.

8.4 Clinic Expenses and Funding

Table 34. Estimated Percentage of Clinic Expenses by Category

Expense Category	n	Mean %	Median %	Min %	Max %
Materials and testing supplies	90	22.9	15	0	100
Clinic staff salaries/stipends	88	20.9	0	0	100
Unknown / cannot estimate	91	15.2	0	0	100
Technology	91	12.1	5	0	90
Director or administrative salaries	90	9.4	0	0	90
Graduate student pay/assistantships	91	6.6	0	0	80
Other	91	3.7	0	0	50
Facilities or overhead	90	2.5	0	0	50

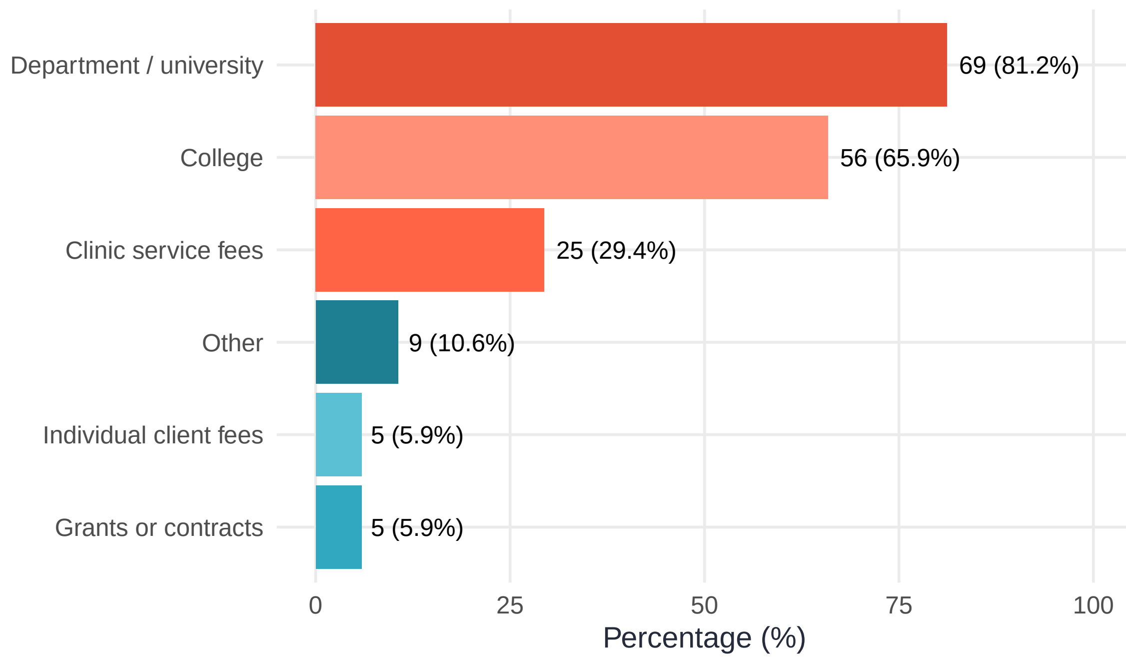

Figure 15. Clinic funding sources. Multiple selections permitted.

Clinics were asked to estimate the percentage of clinic expenses allocated to each category, with totals intended to sum to approximately 100%. Among clinics providing expense allocation data ($n = 91$), 80 had totals between 95% and 105%, 8 had totals below 95%, and 3 had totals above 105%. These values should therefore be interpreted as approximate expense allocations rather than audited budget data.

8.5 Clinic Revenue

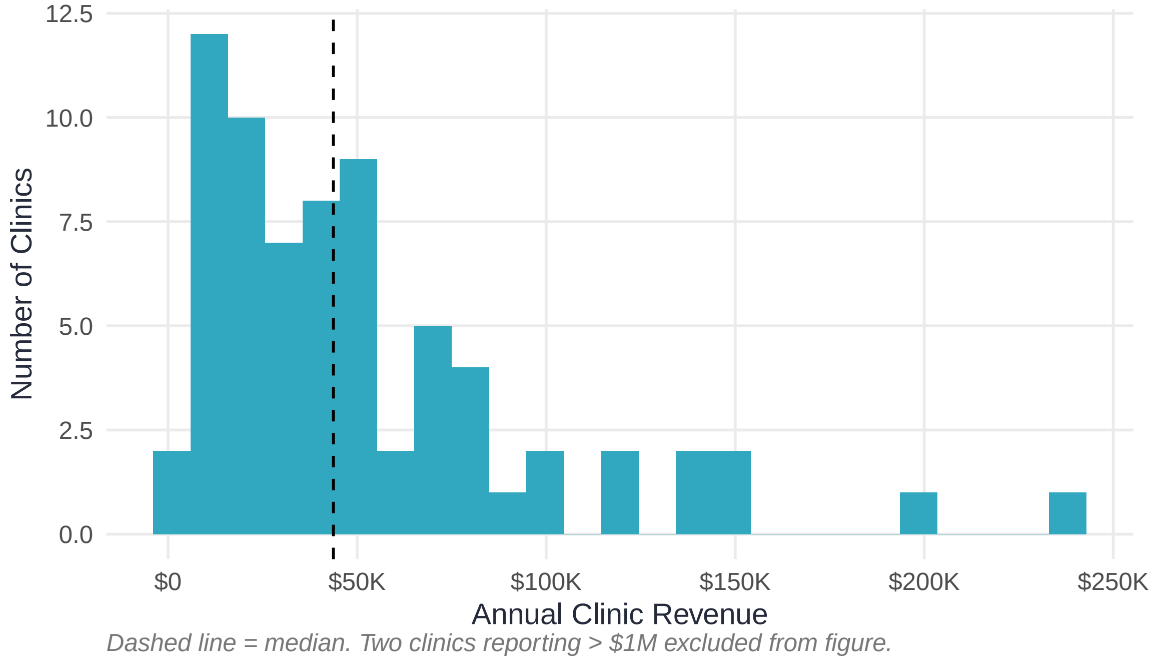

Figure 16. *Annual clinic revenue distribution (clinics with revenue > $0 and < $500,000 shown; outliers excluded from visualization but included in summary statistics).*

Among clinics reporting revenue, the median annual revenue was $43,726 (M = $105,626; $n = 74$). Revenue was strongly associated with clinic size. Because clinic size is a primary stratification variable in this report, size definitions are repeated here: Small clinics served fewer than 93 clients/year, Medium clinics served 93–130 clients/year, Large clinics served 131–250 clients/year, and Giant clinics served more than 250 clients/year.

Table 35. *Annual Revenue by Clinic Size Tier. Small < 93 clients/year, Medium = 93–130 clients/year, Large = 131–250 clients/year, Giant > 250 clients/year. Cells suppressed if n < 3.*

ClinicSize	n	Median Revenue	Mean Revenue
Small	25	$19,500	$23,938
Medium	9	$50,000	$57,956
Large	18	$47,068	$120,085
Giant	16	$88,000	$265,229

Table 36. *Therapy and Assessment Revenue by Clinic Size Tier. Small < 93 clients/year, Medium = 93–130 clients/year, Large = 131–250 clients/year, Giant > 250 clients/year. Cells suppressed if n < 3.*

ClinicSize	Domain	n	Median Revenue	Mean Revenue
Small	Assessment Revenue	17	$10,500	$14,364
Small	Therapy Revenue	17	$8,000	$9,846
Medium	Assessment Revenue	7	$13,700	$26,600
Medium	Therapy Revenue	6	$37,700	$37,467
Large	Assessment Revenue	10	$17,920	$25,909
Large	Therapy Revenue	11	$22,850	$26,463
Giant	Assessment Revenue	9	$60,000	$60,799
Giant	Therapy Revenue	11	$55,000	$147,404

9 Technology

9.1 Electronic Health Records

70% of clinics ($n = 84$) reported using an electronic health record (EHR/EMR) system.

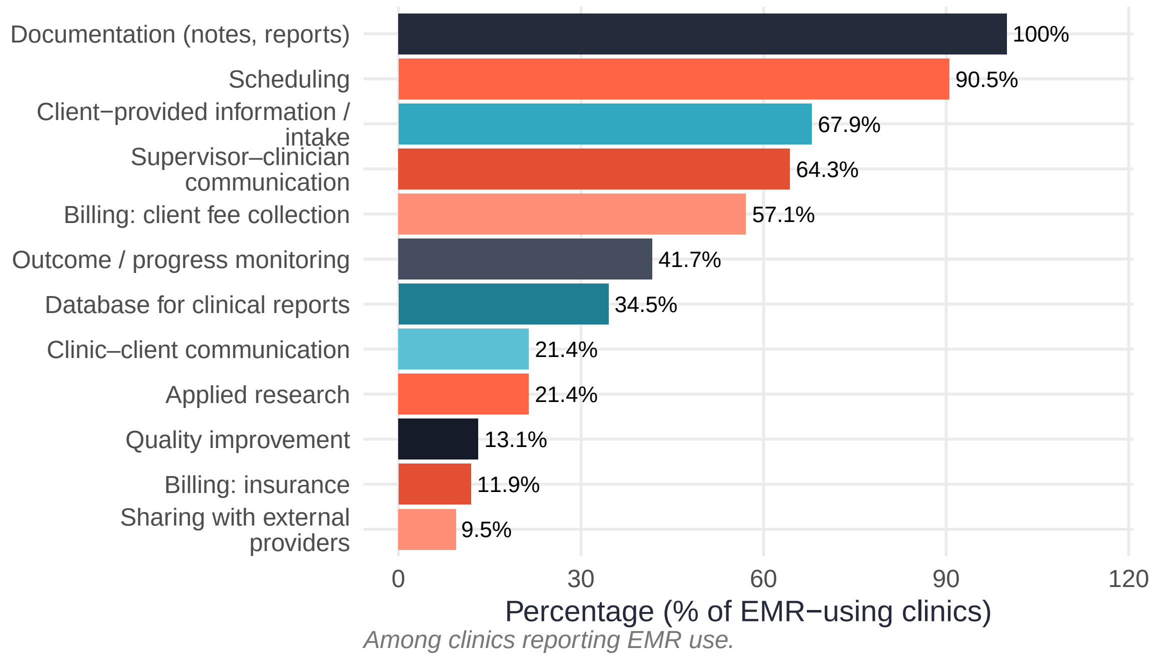

Figure 17. *Functions for which EHR/EMR systems are used. Multiple selections permitted.*

9.2 Routine Outcome Monitoring

58.3% of clinics ($n = 70$) reported using some form of routine outcome monitoring (ROM).

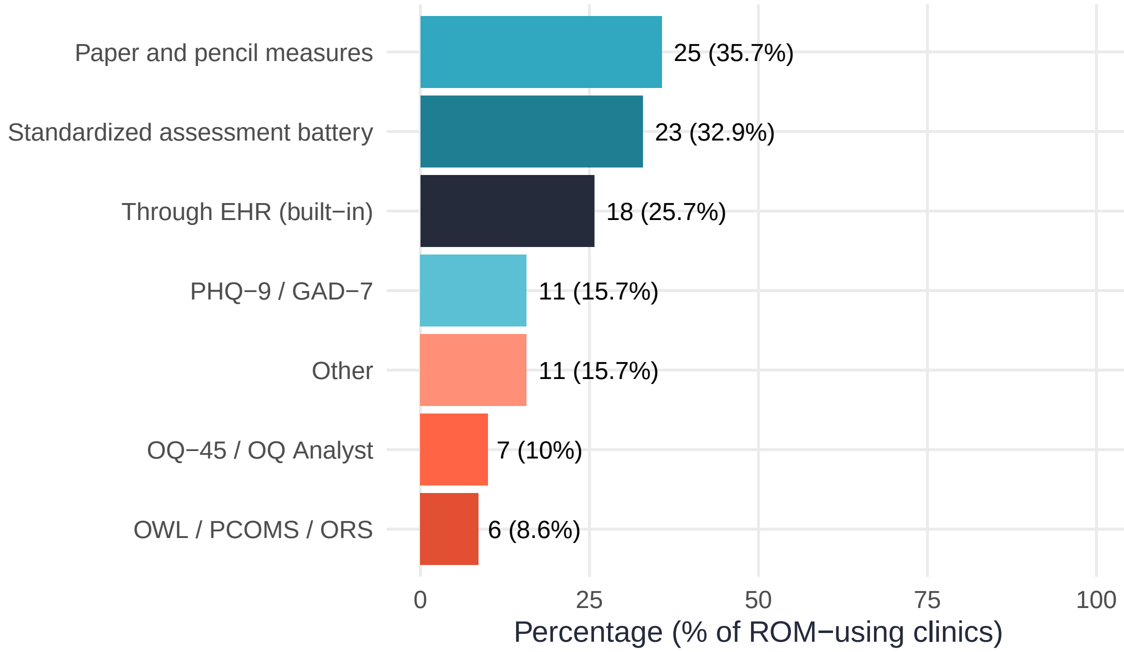

Figure 18. Routine outcome monitoring systems in use. Multiple selections permitted.

9.3 Telehealth

70.8% of clinics ($n = 85$) offer telehealth services in some form.

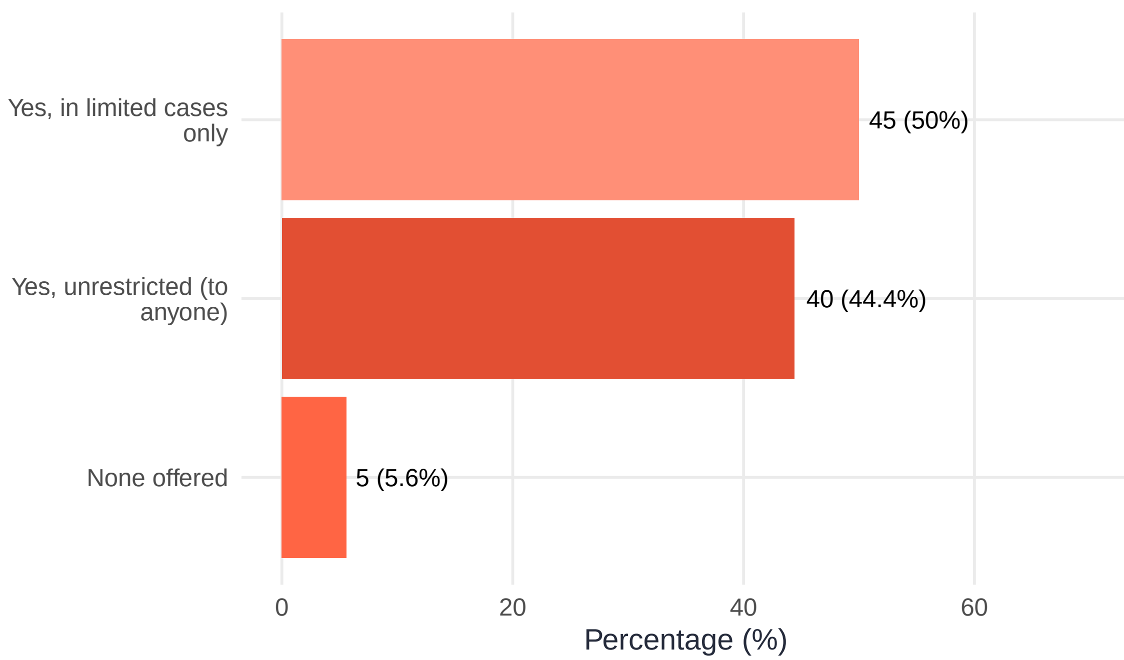

Figure 19. Telehealth availability across APTC clinics.

Table 37. Service Delivery Modality by Service Type: Percentage of Sessions

Service	n	Mean % In-Person	Mean % Telehealth	Median % Telehealth
Assessment	83	91.8	8.2	1
Clinical Supervision	91	78.4	21.6	10
Therapy	84	75.4	24.6	20

9.4 Artificial Intelligence

25.3% of clinic directors ($n = 23$) reported personally using AI in their work. Among clinics responding to the student AI use question, 4.4% explicitly allow student AI use; the majority reported that student AI use had not yet been formally addressed. Formal AI policies are in place at 76.9% of clinics that responded to the policy question.

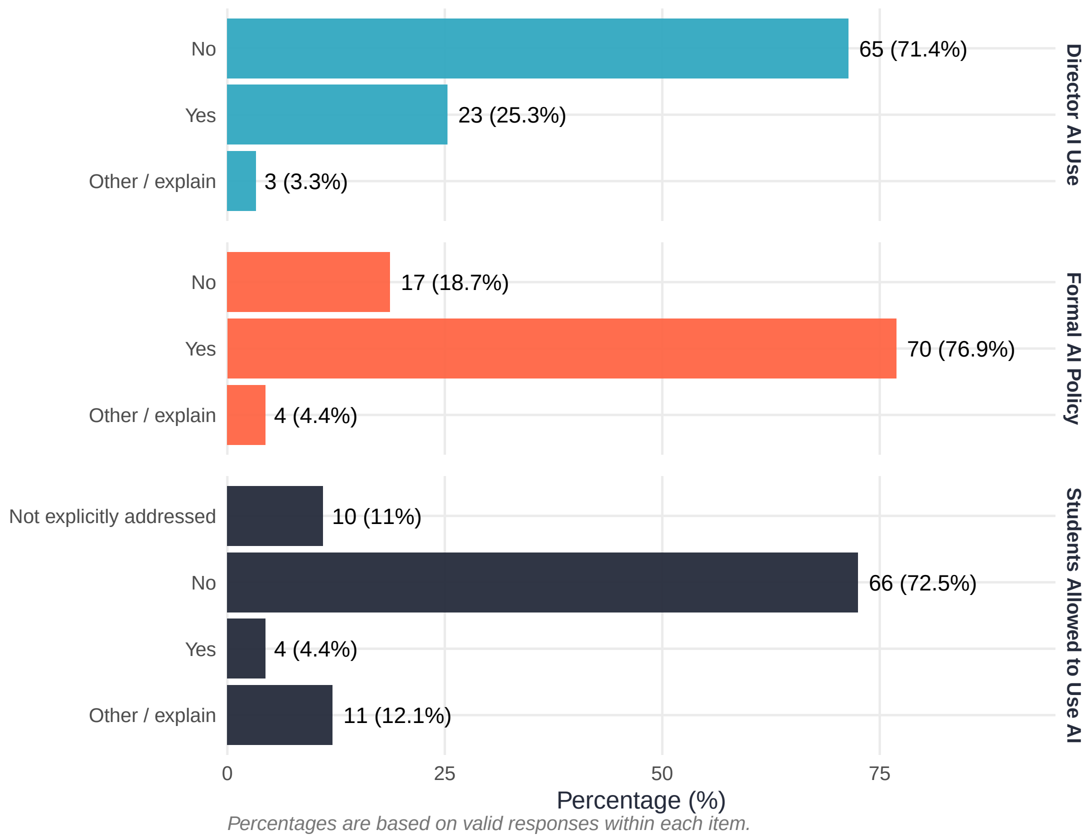

Figure 20. *AI use and policy status across APTC clinics.*

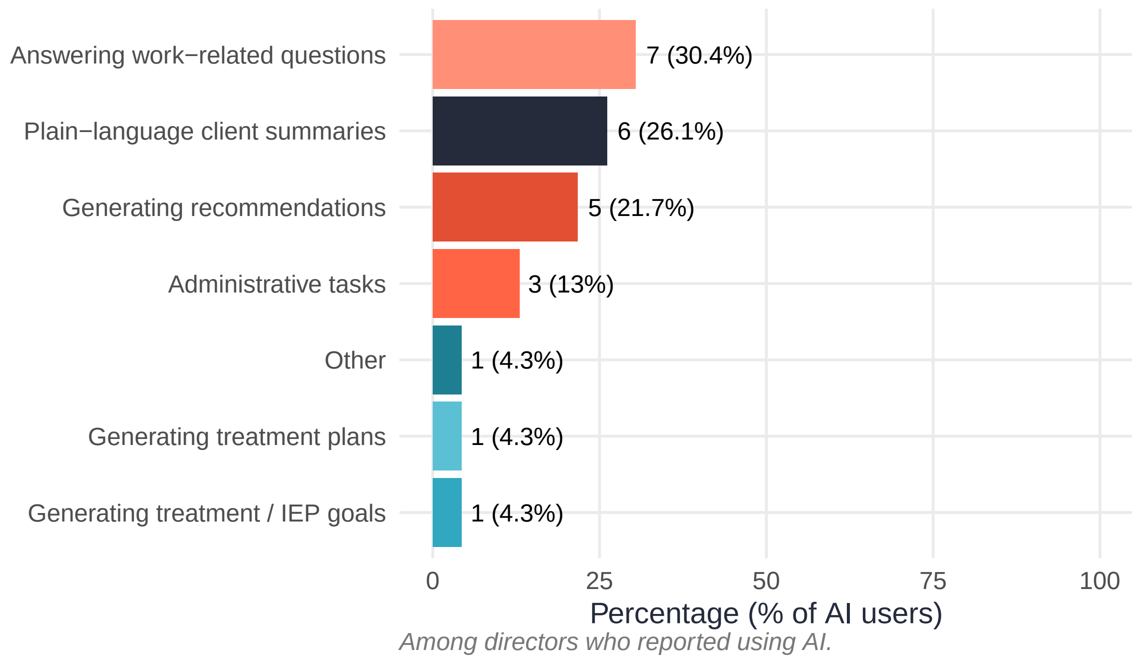

Figure 21. *Specific uses of AI reported by clinic directors. Among those who reported using AI.*

10 Diversity, Equity, and Inclusion

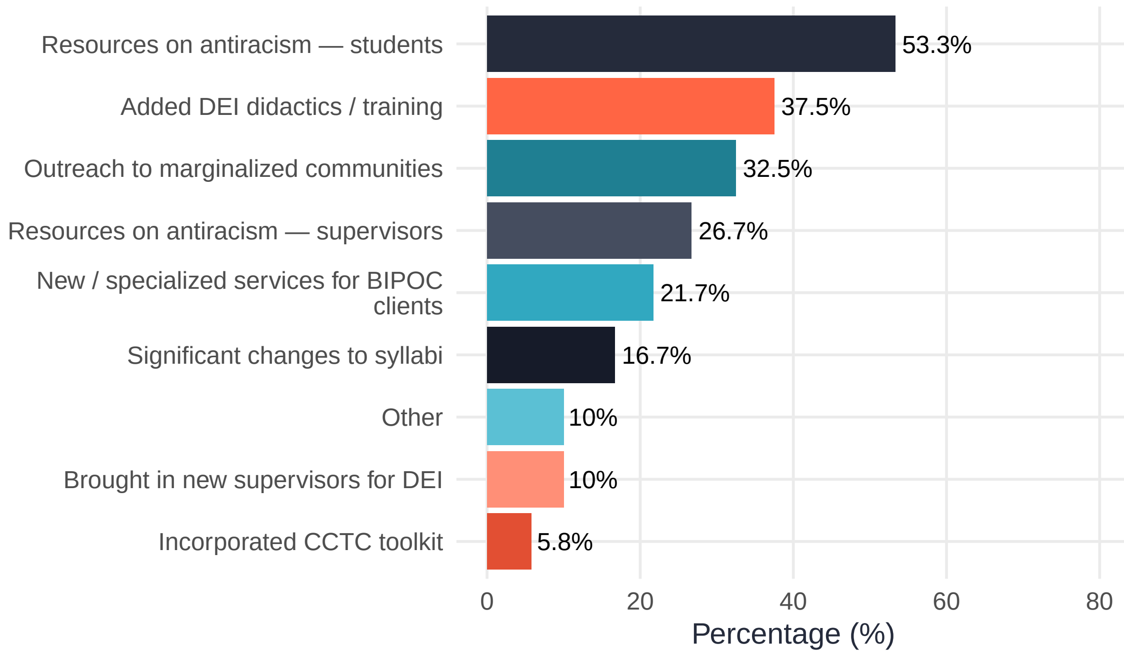

Figure 22. *Social justice and DEI activities undertaken by clinics in the past year. Multiple selections permitted.*

11 Research Activities

Among respondents who answered this item, 48.4% (n = 44 of 91) reported that their clinic is actively engaged in clinic-based research.

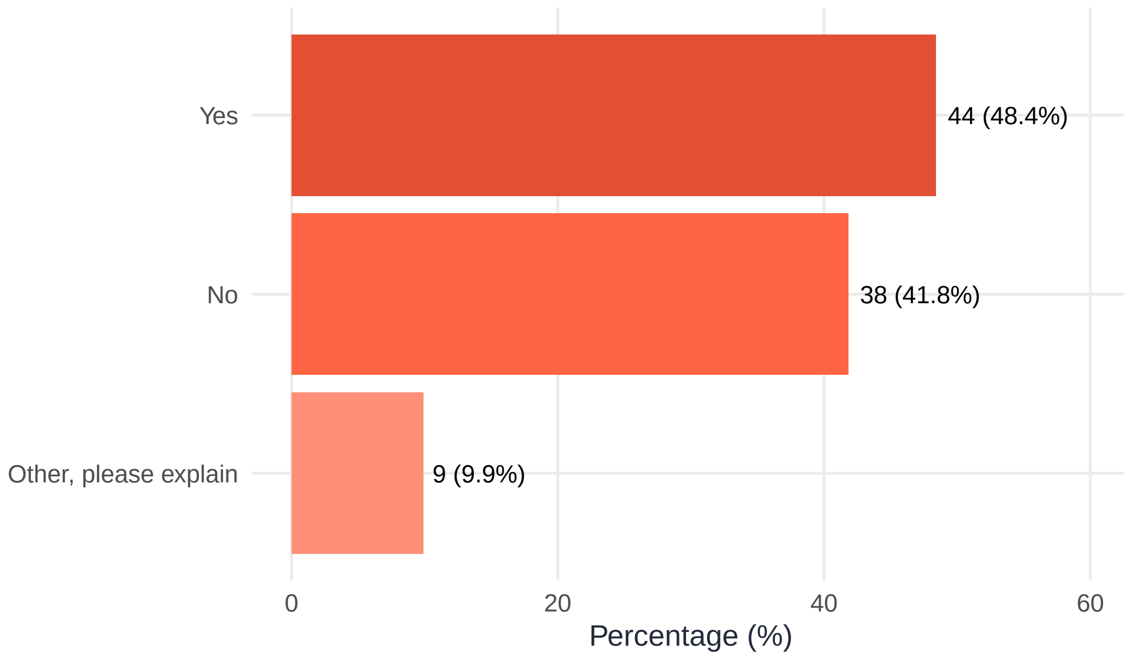

Figure 23. *Research activity status in APTC training clinics.*

12 Director Well-Being and Burnout

12.1 Burnout

Burnout was assessed using five items from the Copenhagen Burnout Inventory (CBI), each rated on a 0–6 scale (higher = greater burnout). The composite burnout score (mean of the five items) had a mean of 2.24 (*SD* = 1.26).

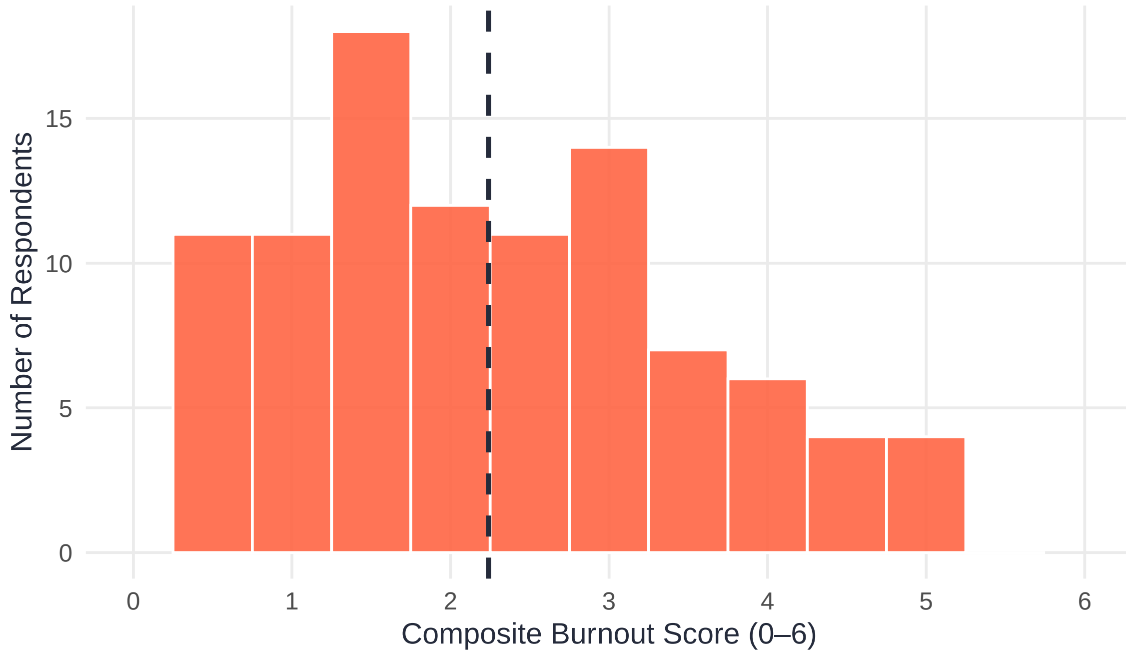

Figure 24. *Distribution of composite burnout scores among clinic directors (0–6 scale; higher = greater burnout). Dashed line = mean.*

Table 38. *Copenhagen Burnout Inventory Item-Level Summary (0–6 scale)*

CBI Item	n	Mean	SD	Median
How often do you feel tired?	100	2.96	1.52	3
How often are you physically exhausted?	100	2.65	1.59	3
Do you feel worn out at the end of the working day?	100	2.27	1.57	2
Do you feel burned out because of your work?	100	1.94	1.58	2
Do you find it hard to work with clients?	100	1.40	1.46	1

12.2 Burnout by Position Type

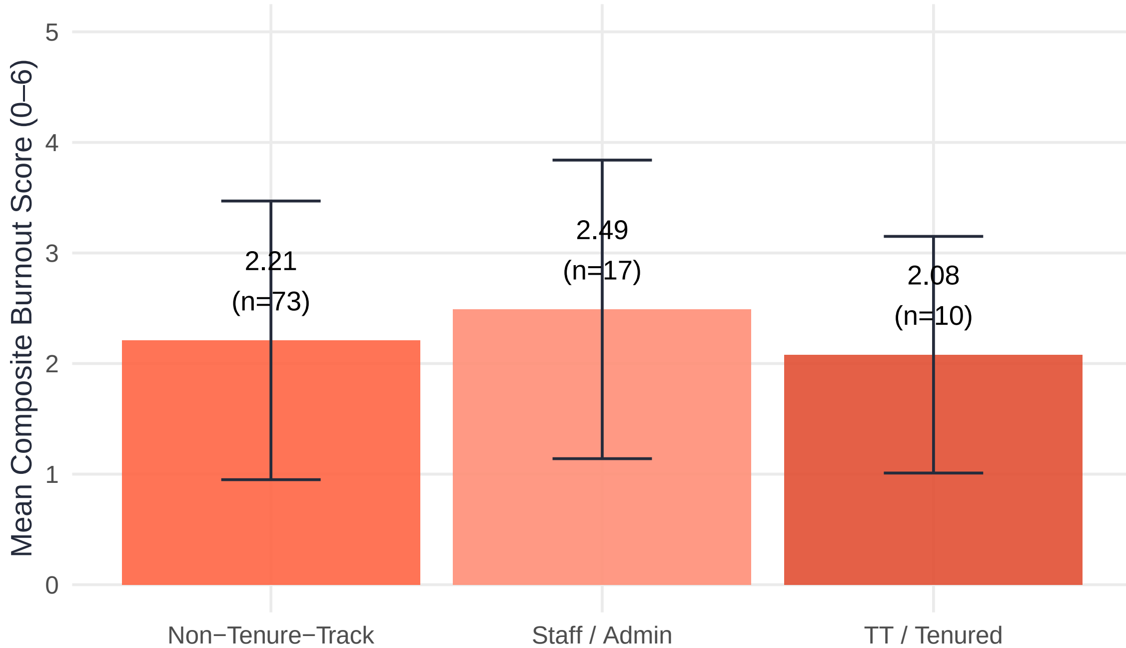

Figure 25. Composite burnout scores by appointment type. Error bars = ±1 SD.

12.3 What Sustains Directors

Directors were asked an open-ended question about what helps them sustain themselves in the role. Common themes included:

- **Colleagues and community** — supportive peers, departmental relationships, and the APTC network itself
- **Autonomy and flexibility** — control over schedule and how clinic work is structured
- **Impact and meaning** — watching students develop professionally and serving underserved communities
- **Clear boundaries** — deliberate separation of work and personal time

12.4 What Directors Find Challenging

Common challenges included administrative burden, lack of institutional support or recognition, student-related stressors, and pay disparities relative to the demands of the role.

> **i** Note on qualitative findings
>
> The themes above are drawn from open-ended responses to items about job positives, negatives, and burnout prevention. These themes are consistent with findings from prior APTC surveys and the 2026 annual conference discussion. Full qualitative summaries are available from the Research Committee upon request.

13 Survey Feedback

13.1 Length and Content

Table 39. *Respondent Feedback on Whether the Survey was Too Long*

Response	n	%
Yes, but the length is necessary	53	53.5%
No	23	23.2%
Yes, it should be shorter	18	18.2%
Other, Please explain:	5	5.1%

Members most frequently requested more detail on student caseloads, stipend and course release data, and clinic-size comparisons. Recurring suggestions for reduction included questions about physical space and detailed year-over-year calculations.

14 Methodological Notes

14.1 Privacy and Cell Suppression

To protect respondent privacy, salary estimates and other sensitive subgroup analyses are suppressed (displayed as "—") when based on fewer than three respondents.

14.2 IRB Status

Survey data collection was approved by the Appalachian State University Institutional Review Board (IRB). The research team is exploring whether future iterations of the survey may not meet the regulatory definition of human subjects research, which would permit more tailored reporting of member data while preserving appropriate safeguards. For example, such a determination could allow benchmarking within more specific peer groups (e.g., public R1 universities in the Northeast or clinics of similar size).

To assess member perspectives on this possibility, APTC members were surveyed in March 2026 regarding potential reductions in data-use restrictions. Of the 31 members who responded, 87.1% (n = 27) supported pursuing a determination that the activity does not constitute human subjects research and indicated comfort with more tailored reporting, including reporting that may be more identifiable in some contexts. Two respondents (6.5%) indicated they were ambivalent or had no preference, and two respondents (6.5%) preferred that responses remain anonymous and subject to the current reporting restrictions.

14.3 Data Source and Respondent Perspective

Clinic-level results are based on self-report from a single respondent per clinic. Although multiple individuals from some clinics participated in the broader survey, clinic-level questions were completed by only one respondent per clinic and therefore reflect that individual's perspective. Individual-level questions (e.g., director demographics, compensation, workload, and professional activities) were analyzed at the respondent level and may include multiple respondents from the same clinic.

14.4 Financial Data

Respondents were instructed to report all financial information in U.S. dollars, and a currency conversion resource was provided within the survey. Financial results are reported as submitted by respondents.

14.5 Limitations

Because not every respondent answered every item, effective sample sizes vary throughout this report. Directors who were new to their role in the survey year may have had limited access to historical clinic data. Clinic-level results should be interpreted as self-reported estimates from the designated clinic respondent. Finally, results are descriptive; causal inferences are not supported.

Acknowledgments

The Research Committee thanks all APTC member directors who completed the 2026 survey. Special thanks to Richard LeBeau (UCLA) for contributions at APTC 2026, and to the broader APTC membership for feedback that continues to shape the survey's development.

Contact

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